RISK MANAGEMENT PLAN
EXECUTIVE OVERVIEW

Training draft based on the uploaded synthetic pharmacovigilance evidence pack

Selected analytical focus: Losartan – Fatigue

Prepared as: Educational RMP exercise | **Evidence:** 2 user-provided CSV files | **Analysis date:** 9 November 2026

Important scope note
The CSV pack contains eight medicine–reaction pairs. Losartan is used as the working focus because the current exercise has examined the Losartan–Fatigue reports. The files do not establish that Losartan is the newly launched product. Confirm the intended product with the Workora task brief before final submission.

Training-use limitation
All records are synthetic. This document is a structured learning draft, not a regulator-ready EU RMP, a clinical safety conclusion, or a substitute for the authorised product information, clinical/non-clinical evidence, company core data sheet, and qualified safety review.

1. Executive summary

This draft reviews a synthetic aggregate dataset of 320 individual adverse-event report records and a synthetic product-reference table covering eight medicines. The selected analytical focus is the reported association of Losartan with fatigue. The dataset contains 38 Losartan–Fatigue reports, all labelled “Unexpected” against the supplied synthetic reference. Three reports are marked serious and 35 non-serious. These observations justify documenting the finding for further evaluation; they do not establish causality, incidence, a confirmed safety signal, or a formal important potential risk.

The proposed approach is to continue routine pharmacovigilance, perform a documented signal validation and medical review, obtain missing case details where possible, and review authoritative product information and the full clinical/non-clinical safety database. Additional studies or additional risk minimisation should be introduced only if the totality of evidence and the benefit-risk assessment justify them.

2. Product and evidence scope

Item	Information available in supplied pack
Working focus	Losartan
Reaction under review	Fatigue
Reference entry	Fatigue – ReferenceStatus: Unexpected
Reference version	Synthetic Core Data Sheet v1.0
Dataset size	320 reports; 8 medicine–reaction pairs
Report date range	2026-01-01 to 2026-08-24
Geographic field	All 320 records list India

The reference file contains one listed reaction for each of eight medicines. It is a synthetic reference only and must not be treated as the approved SmPC, Package Leaflet, RSI, or current company Core Data Sheet.

3. Safety data analysis – Losartan and fatigue

3.1 Report counts

Measure	Count
Total Losartan–Fatigue reports	38
Serious	3
Non-serious	35
Unexpected	38
Known	0

The 38 reports represent 11.9% of the 320 synthetic records. This is only the proportion of records in this constructed dataset; it is not an incidence rate or a population risk estimate because exposure denominators and sampling methods are not provided.

3.2 Reported outcomes

Outcome	Reports
Not recovered	12
Unknown	11
Recovered	8
Recovering	7

No records in this selected group are labelled Fatal or Recovered with sequelae. “Unknown” and “Not recovered” outcomes indicate that follow-up or clinical status may remain incomplete; they do not by themselves indicate a causal relationship.

3.3 Other case characteristics

Characteristic	Observed in the 38 records
Age	18–85 years; mean approximately 48.1 years
Reporter type	19 pharmacists; 12 consumers; 7 physicians
Dechallenge	17 positive; 15 negative; 6 unknown
Rechallenge	12 positive; 10 negative; 16 not done
Seriousness criteria	2 other medically important conditions; 1 hospitalisation; 35 not serious
Country	India in all 38 records

A positive dechallenge or rechallenge entry is a data field requiring case-level clinical review. The dataset does not provide enough narrative detail to independently validate those assessments or exclude alternative causes.

4. Safety specification – provisional assessment

The following is a cautious training assessment. Formal RMP classification requires review of the complete safety database, clinical trial data, non-clinical findings, literature, epidemiology, product information, and medical judgement.

4.1 Important identified risks

No Important Identified Risk can be confirmed from these two CSV files alone. The reference marks Losartan–Fatigue as unexpected, and spontaneous/synthetic report counts do not establish causality. Do not label fatigue an identified risk solely on the basis of these reports.

4.2 Important potential risks

Losartan–Fatigue may be recorded as a **potential safety topic requiring signal evaluation**. The 38 reports, including three marked serious and a mixture of dechallenge/rechallenge entries, warrant review. However, this dataset alone is insufficient to classify fatigue formally as an Important Potential Risk. A signal validation should examine temporal relationship, dose, indication, concomitant medicines, medical history, alternative causes, clinical plausibility, duplicate reports, and external evidence.

4.3 Missing information

The supplied CSVs do not describe the product’s target populations or the completeness of its clinical safety database. Potential information gaps to assess against the actual development and post-authorisation evidence include use during pregnancy and breastfeeding, use in children/adolescents if relevant, long-term exposure, and use in patients with significant renal or hepatic impairment. These are **proposed review topics only**, not confirmed missing information for this product. Include a gap in the RMP only when the available evidence demonstrates that safety information is insufficient for a clinically relevant population or use.

5. Pharmacovigilance plan

5.1 Routine pharmacovigilance

• Collect, validate, medically review, code, and report suspected adverse reaction cases in accordance with applicable requirements and company procedures.
• Conduct ongoing signal detection and review cumulative safety data, including seriousness, expectedness, outcomes, and case narratives.
• Perform routine medical literature monitoring and review relevant regulatory safety information.
• Prepare periodic safety reports and maintain an up-to-date benefit-risk evaluation as required by the applicable regulatory framework.
• Maintain case follow-up, data quality checks, duplicate detection, and documented quality control.

5.2 Additional pharmacovigilance

No additional study is justified by this synthetic dataset alone. If signal validation and the broader evidence review identify an unresolved safety question, consider a targeted post-authorisation safety study (PASS), drug-utilisation study, registry, or enhanced follow-up. The objective, population, endpoints, feasibility, and regulatory status should be defined before proposing any study. Do not list a study as committed unless it has been scientifically justified and approved through the appropriate process.

6. Risk minimisation measures

6.1 Routine measures

Maintain accurate and current product information, including warnings and precautions supported by the approved evidence. Ensure healthcare professionals and patients receive the approved package leaflet and prescribing information. Any wording change must be based on validated evidence and follow the relevant regulatory variation process.

6.2 Additional measures

A patient alert card or educational brochure is **not recommended automatically** for fatigue on the basis of these reports. Additional measures should be proposed only if the risk is confirmed as important, routine measures are insufficient, and the intervention is proportionate. If required after assessment, design targeted materials for the relevant audience and evaluate whether they improve knowledge or safer use.

7. Evaluation of effectiveness

If additional risk minimisation is introduced, define measurable objectives before implementation. Possible evaluation methods include healthcare-professional/patient knowledge surveys, review of correct use and adherence to recommended actions, process indicators showing material distribution and reach, and outcome indicators where scientifically appropriate. Set a baseline, target, evaluation period, data source, and responsible owner. A distribution count alone does not prove that a measure reduced harm.

8. Overall conclusion and actions before final submission

The synthetic evidence pack contains 38 Losartan–Fatigue reports, all labelled unexpected in the supplied synthetic reference, including three reports marked serious. These data support documenting and evaluating the topic, but do not establish causality or justify a definitive Important Identified Risk or Important Potential Risk classification. Routine pharmacovigilance and a structured signal review are appropriate next steps for this training scenario. Additional studies and additional risk minimisation should depend on the complete evidence review.

Before submitting to Workora, complete these checks:

1. Confirm that Losartan is the intended newly launched product in the task brief. The CSV pack itself contains eight products and does not identify which is newly launched.
2. Replace the synthetic reference with the actual approved product information/RSI and relevant clinical safety evidence, if provided in the task pack.
3. Verify the product name, active substance, strength, formulation, indication, authorisation date, applicant/MAH, and target jurisdiction from the source documents; these details are not present in the CSV files.
4. Confirm whether Workora expects an executive overview only or a full EU RMP template.
5. Have the final classification and measures reviewed by the assignment supervisor/qualified PV reviewer.

Regulatory framing: This document is structured around the general EU GVP RMP concepts (safety specification, pharmacovigilance plan, risk minimisation and evaluation). It is not a claim of formal regulatory compliance or a complete EU RMP submission.

Appendix – Dataset inventory

Medicine	Reaction	Reference status	Reports
Amlodipine	Peripheral oedema	Known	51
Amoxicillin	Rash	Known	39

Medicine	Reaction	Reference status	Reports
Atorvastatin	Myalgia	Known	46
Ciprofloxacin	Tendon pain	Known	35
Losartan	Fatigue	Unexpected	38
Metformin	Nausea	Known	33
Omeprazole	Headache	Known	42
Sertraline	Dizziness	Known	36

Source files analysed: aggregate_safety_reports(1).csv (320 rows, 18 columns) and synthetic_product_reference(1).csv (8 rows, 4 columns). Analysis is descriptive and based only on the supplied synthetic fields.